Which treatment this guide covers This guide covers NCCP 00353 osimertinib on its own: T790M-positive advanced disease, first-line activating-EGFR advanced disease, or completely resected stage IB–IIIA Ex19del/L858R adjuvant treatment. The newer licensed post-chemoradiation setting and osimertinib with platinum/pemetrexed need their own applicable plan; their study results and support medicines are not added to this monotherapy suite.

NCCP 00353 · VERSION 5

Osimertinib monotherapy

The usual starting dose is 80 mg by mouth once daily, every day without a planned rest week. Use the dose on your prescription. Advanced treatment continues while helping and tolerated; adjuvant treatment is planned for up to three years, with earlier stopping if cancer returns or side effects become unacceptable.

Oral targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Eye pain or blurred vision

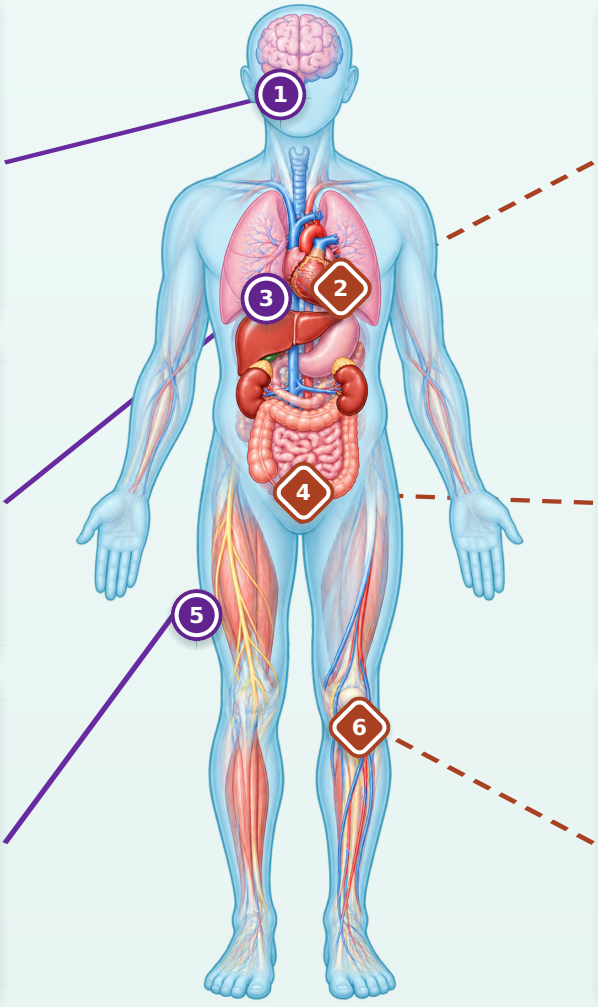
osimertinib

Possible keratitis needs prompt specialist advice.

2 New cough or breathlessness

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Possible lung inflammation needs urgent assessment.



3 Palpitations or fainting

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Heart rhythm or pumping changes need assessment.

4 Diarrhoea

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Report early; severe symptoms or dehydration need urgent advice.

5 Skin and nail changes

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Blistering, peeling or mucosal sores are urgent.

6 Bruising or bleeding

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Blood-count changes require tests and clinical review.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team urgently for new or worsening breathlessness, cough or fever; fainting, palpitations or chest pain; blistering/peeling skin or mouth/eye/genital sores; unusual bruising, bleeding or marked pallor; severe diarrhoea, dehydration or inability to drink. New eye pain, light sensitivity or blurred vision needs prompt advice. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

Know the next step.

Taking treatment

The usual starting dose is 80 mg by mouth once daily, every day without a planned rest week. Use the dose on your prescription. Advanced treatment continues while helping and tolerated; adjuvant treatment is planned for up to three years, with earlier stopping if cancer returns or side effects become unacceptable.

Take at the same time each day, with or without food. Swallow whole with water; do not crush, split or chew. If you forget a dose, take it when remembered unless less than 12 hours remain before the next dose, when it should be skipped. Confirm the instruction with your pharmacist if the next dose is exactly 12 hours away. Do not take an extra dose without advice if you vomit. Ask for the approved water-dispersion instructions if swallowing is difficult.

Checks and follow-up

Confirm the relevant EGFR test, treatment setting and prescription. Adjuvant treatment needs complete resection and the specific stage/mutation criteria. Blood count, kidney, liver and bone profile; ECG before treatment. Consider an echocardiogram or MUGA LVEF assessment for cardiac risks or symptoms; review all medicines, pregnancy, contraception and breastfeeding. Repeat blood count, kidney, liver and bone profile at the agreed reviews; extra checks may be needed for symptoms or abnormal results. NCCP specifies ECG at baseline and cycle 2, then cycle 3 onwards if clinically indicated. The team translates cycle 2 into a booked review; the tablets remain daily, without a treatment break. LVEF assessment if clinically indicated; ECG and electrolytes with cardiac/QT risks. Scans and disease checks follow the Consultant plan.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Skin and nails

Agree an EGFR-inhibitor skin-care plan from the start and report rash or painful nail changes early. Blistering, peeling or mucosal sores need urgent assessment.

Bowel symptoms and hydration

Report diarrhoea early, follow the prescribed symptom plan and maintain fluids if able. Severe diarrhoea, weakness or reduced urine needs urgent advice.

Nausea medicines

Osimertinib has minimal-to-low oral emetic risk. Routine infusion premedication is not usually needed. Agree rescue medicines individually, including QT and interaction review; do not copy a chemotherapy antiemetic course.

Medicine interactions

Tell the team about all medicines and supplements. Do not take St John’s wort. Strong enzyme-inducing medicines and medicines that affect heart rhythm require pharmacy review before changes.

Pregnancy, fertility and breastfeeding

Avoid pregnancy during treatment. Effective contraception continues for at least two months after the final dose for women and four months for men. Osimertinib may reduce hormonal contraceptive effectiveness; discuss a suitable method. Do not breastfeed during treatment. Discuss fertility and plans for pregnancy before starting.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital’s urgent-care plan. Emergency: 112 / 999.

Your complete treatment reference

https://sactcheck.com/patient/00353/

Public information only. No personal information is encoded.